

Research on the common ethical issues and legislative breaches in the social and intermediate long-term care sector

Ethical Issues and Legislative breaches

Legislative breach - PDPA

SingHealth's IT System target of Cyberattack

<https://www.moh.gov.sg/newsroom/singhealth's-it-system-target-of-cyberattack/>

Further reading

Public Report of the COI into the cyber attack on Singapore Health Patient Database

[Public Report of the COI into the cyber attack on Singapore Health Patient Database](#)

[PUBLIC REPORT OF THE COMMITTEE OF INQUIRY INTO THE CYBER ATTACK ON SINGAPORE HEALTH SERVICES PRIVATE LIMITED'S PATIENT DATABA](#)

Cyberattack on SingHealth IT System - Information for SingHealth patients

[Data Security Check | SingHealth](#)

Joint Press Release by MCI and MOH - SingHealth's IT System Target of Cyberattack

<https://www.singhealth.com.sg/news/joint-press-release-by-mci-and-moh-singhealths-it-system-target-of-cyberattack>

Legislative Breach - Negligence Duty of Care

Death caused by a surgeon's "scandalous and deplorable" conduct in diagnosis, treatment and aftercare

Chong Khin Ngen v Lim Djoe Phing [1993] SGHC 154 (Biomedical Ethics in Singapore, page 95)

The Singapore case of Pang Koi Fah v Lim Djoe Phing(18) involved a claim for "nervous shock" to a third party arising out of medical negligence. In that case, the mother of a young lady was wrongly told by a neurosurgeon that, if her daughter did not have an operation immediately, she would die. For obvious reasons, the mother consented to the operation, which went terribly wrong. The neurosurgeon removed healthy tissue and in the process caused a tear in the patient's arachnoid membrane. The patient began to leak cerebro - spinal fluid through her nose and then developed meningitis and ultimately passed away after much pain and suffering, witnessed by her mother.

[Medical Negligence: Duty of Care](#)

Key Reference

Biomedical Ethics in Singapore - Cases and Commentary 2025

[biomedical ethics in singapore: cases and commentary](#)

Singapore Statues
Personal Data Protection Act 2012

<https://sso.agc.gov.sg/Act/PDPA2012?ViewType=Advance&Phrase=personal+data+protection&WiAI=1&ProvIds=P16A-#pr26E->

Part 6A - Notification of Data Breaches

Interpretation of this Part

26A. In this Part, unless the context otherwise requires —

“affected individual” means any individual to whom any personal data affected by a data breach relates;

“data breach”, in relation to personal data, means —

- (a) the unauthorised access, collection, use, disclosure, copying, modification or disposal of personal data; or
- (b) the loss of any storage medium or device on which personal data is stored in circumstances where the unauthorised access, collection, use, disclosure, copying, modification or disposal of the personal data is likely to occur.

Notifiable data breaches

26B.—(1) A data breach is a notifiable data breach if the data breach —

- (a) results in, or is likely to result in, significant harm to an affected individual; or
- (b) is, or is likely to be, of a significant scale.

(2) Without limiting subsection (1)(a), a data breach is deemed to result in significant harm to an individual —

- (a) if the data breach is in relation to any prescribed personal data or class of personal data relating to the individual; or
- (b) in other prescribed circumstances.

(3) Without limiting subsection (1)(b), a data breach is deemed to be of a significant scale —

- (a) if the data breach affects not fewer than the prescribed number of affected individuals; or
- (b) in other prescribed circumstances.

(4) Despite subsections (1), (2) and (3), a data breach that relates to the unauthorised access, collection, use, disclosure, copying or modification of personal data only within an organisation is deemed not to be a notifiable data breach.

Duty to conduct assessment of data breach

26C.—(1) This section applies to a data breach that occurs on or after 1 February 2021.

(2) Subject to subsection (3), where an organisation has reason to believe that a data breach affecting personal data in its possession or under its control has occurred, the organisation must conduct, in a reasonable and expeditious manner, an assessment of whether the data breach is a notifiable data breach.

(3) Where a data intermediary (other than a data intermediary mentioned in section 26E) has reason to believe that a data breach has occurred in relation to personal data that the data intermediary is processing on behalf of and for the purposes of another organisation —

(a) the data intermediary must, without undue delay, notify that other organisation of the occurrence of the data breach; and

(b) that other organisation must, upon notification by the data intermediary, conduct an assessment of whether the data breach is a notifiable data breach.

(4) The organisation must carry out the assessment mentioned in subsection (2) or (3)(b) in accordance with any prescribed requirements.

Duty to notify occurrence of notifiable data breach

26D.—(1) Where an organisation assesses, in accordance with section 26C, that a data breach is a notifiable data breach, the organisation must notify the Commission as soon as is practicable, but in any case no later than 3 calendar days after the day the organisation makes that assessment.

(2) Subject to subsections (5), (6) and (7), on or after notifying the Commission under subsection (1), the organisation must also notify each affected individual affected by a notifiable data breach mentioned in section 26B(1)(a) in any manner that is reasonable in the circumstances.

(3) The notification under subsection (1) or (2) must contain, to the best of the knowledge and belief of the organisation at the time it notifies the Commission or affected individual (as the case may be), all the information that is prescribed for this purpose.

(4) The notification under subsection (1) must be made in the form and submitted in the manner required by the Commission.

(5) Subsection (2) does not apply to an organisation in relation to an affected individual if the organisation —

(a) on or after assessing that the data breach is a notifiable data breach, takes any action, in accordance with any prescribed requirements, that renders it unlikely that the notifiable data breach will result in significant harm to the affected individual; or

(b) had implemented, prior to the occurrence of the notifiable data breach, any technological measure that renders it unlikely that the notifiable data breach will result in significant harm to the affected individual.

(6) An organisation must not notify any affected individual in accordance with subsection (2) if —

(a) a prescribed law enforcement agency so instructs; or

(b) the Commission so directs.

(7) The Commission may, on the written application of an organisation, waive the requirement to notify an affected individual under subsection (2) subject to any conditions that the Commission thinks fit.

(8) An organisation is not, by reason only of notifying the Commission under subsection (1) or an affected individual under subsection (2), to be regarded as being in breach of —

- (a) any duty or obligation under any written law or rule of law, or any contract, as to secrecy or other restriction on the disclosure of information; or
- (b) any rule of professional conduct applicable to the organisation.

(9) Subsections (1) and (2) apply concurrently with any obligation of the organisation under any other written law to notify any other person (including any public agency) of the occurrence of a data breach, or to provide any information relating to a data breach.

Obligations of data intermediary of public agency

26E. Where an organisation —

- (a) is a data intermediary processing personal data on behalf of and for the purposes of a public agency; and
- (b) has reason to believe that a data breach has occurred in relation to that personal data, the organisation must, without undue delay, notify the public agency of the occurrence of the data breach.

Reference

[Personal Data Protection Act 2012 - Singapore Statutes Online](#)

Medical Registration Act 1997 (MRA): Governs doctor registration and professional conduct, with the SMC handling complaints.

Voluntary removal, suspension, etc.

38.—(1) A registered medical practitioner may request the Medical Council to take one or more of the actions in subsection (2) if the registered medical practitioner believes that —

- (a) his fitness to practise medicine is impaired by reason of his physical or mental condition; or
- (b) the quality of the professional services provided by him does not meet the standard which is reasonable to expect of a medical practitioner.

(2) Upon receiving a request under subsection (1), the Medical Council may, with the agreement of the registered medical practitioner, do one or more of the following:

- (a) remove the registered medical practitioner's name from the appropriate register;
- (b) suspend the registration of the registered medical practitioner in the appropriate register for a period not exceeding 3 years;
- (c) where the registered medical practitioner is a fully registered medical practitioner in Part I of the Register of Medical Practitioners — remove the registered medical practitioner's name from Part I of that Register and register him instead as a medical practitioner with conditional registration in Part II of that Register, and section 21(4), (6), (7), (8) and (9) applies accordingly;
- (d) where the registered medical practitioner is registered in any register other than Part I of the Register of Medical Practitioners — impose additional appropriate conditions or restrictions

on his registration or vary the conditions or restrictions already imposed under section 21, 23 or 24;

(e) suspend or cancel the registered medical practitioner's practising certificate.

(3) However, the Medical Council must not take any action under subsection (2) in relation to a registered medical practitioner if —

(a) the Medical Council believes that there is evidence of any matter mentioned in section 40(3)(a) or (4)(a), (b) or (c); or

(b) An inquiry under Division 2 has started and is pending against the registered medical practitioner.

(4) Subsections (2) and (3) also apply where the Medical Council has given a notification to the registered medical practitioner under section 40(6)(b), except that if the Medical Council and the registered medical practitioner are unable to agree on the course of action to be taken under subsection (2)(a) to (e), the Medical Council must proceed to refer the matter to the chairman of the Complaints Panel under section 40(6)(a).

[Act 34 of 2020 01/07/2022]

Division 2 — Inquiries into complaints against and
information about registered medical practitioners

Appointment of Complaints Panel

39.—(1) For the purpose of enabling Inquiry Committees, Complaints Committees, Review Committees, Disciplinary Tribunals and Interim Orders Committees to be constituted in accordance with this Part, the Medical Council must appoint a panel (called in this Act the Complaints Panel) consisting of —

(a) at least 10 members of the Medical Council;

(b) at least 10 registered medical practitioners each of whom is of at least 8 years' standing and is not a member of the Medical Council; and

(c) at least 6 other persons nominated by the Minister, each of whom is either —

(i) a legal professional; or

(ii) a lay person.

(2) The term of office of a member of the Complaints Panel mentioned in subsection (1)(a) expires at the end of the member's term of office as a member of the Medical Council.

(3) A member of the Complaints Panel mentioned in subsection (1)(b) or (c)(i) or (ii) is appointed for a term of 3 years and is eligible for re-appointment.

(4) The Medical Council may at any time remove from office any member of the Complaints Panel or fill any vacancy in its membership.

(5) The Medical Council must appoint, from among the members of the Complaints Panel who are members of the Medical Council, the chairman and the deputy chairman of the Complaints Panel.

(6) The chairman and deputy chairman appointed under subsection (5) must be registered medical practitioners.

[Act 34 of 2020 01/07/2022]

Complaints against registered medical practitioners, etc.

40. --(1) A person (called in this Part the complainant) may make a complaint against, or provide information about, any registered medical practitioner to the Medical Council on any matter mentioned in subsection (3) or (4).

- (2) Every complaint against, or information about, any registered medical practitioner mentioned in subsection (1) must —
- (a) be in writing;
 - (b) be supported by a statutory declaration unless the complaint or information is made or provided by a public officer or the Medical Council; and
 - (c) be accompanied by every relevant document and information that is in the possession of the complainant.
- (3) The complaint mentioned in subsection (1) is a complaint —
- (a) relating to —
 - (i) the conduct of a registered medical practitioner in his professional capacity; or
 - (ii) a registered medical practitioner's improper act or conduct which brings disrepute to his profession; or
 - (b) that the professional services provided by a registered medical practitioner are not of the quality that is reasonable to expect of him.
- (4) The information mentioned in subsection (1) is information relating to —
- (a) the conviction (whether in Singapore or elsewhere) of a registered medical practitioner of an offence implying a defect in character that makes him unfit to practise medicine;
 - (b) the death of any patient resulting from the conduct of a registered medical practitioner in his professional capacity;
 - (c) an adverse finding against a registered medical practitioner by a Coroner at an inquiry under the Coroners Act 2010* into the death of a patient of the registered medical practitioner; or
 - (d) the physical or mental fitness of a registered medical practitioner to practise medicine.

Reference

[Medical Registration Act 1997 - Singapore Statutes Online](#)

Ethical Breaches

Autonomy

The Importance of written consent forms in recording of patients' informed consent

Lam Kwok Tai Leslie v Singapore Medical Council [2017] 5 SLR 1169 (Biomedical Ethics in Singapore, page 27)

Justice

The idea of distributive justice: **the fair and equitable distribution of limited healthcare resources across any given population.** The questions accompanying any discussion of justice can be very interesting: for instance, should we de-prioritise the use of medical resources for cigarette smokers in the cancer ward, on the basis that they have “brought this upon themselves”?

For a country with limited healthcare expertise and a significant aging population, it will only be a matter of time before Singapore has to grapple with questions of distributive justice in very real terms.

Patient Autonomy and Informed Consent

How does the need to respect a patient's autonomy be balanced against the professional obligations of a doctor?

Elaine Congress Ethical Decision-Making Model

The problem

The Disciplinary Tribunal convicted Doctor L of the third charge of professional misconduct under Section 53(1)(d) of the MRA for having **failed to obtain informed consent** from P before carrying out the PCI in breach of Guideline 4.2.2 of the SMC's Ethical Code and Ethical Guidelines 2002 ("2002 ECEG").

Nature of the Dilemma

Dignity and Worth of the Person - We want to protect the client's right to decide his life. "Social workers may limit clients' right to self-determination when, in the social workers' professional judgement, clients' actions or potential actions pose a serious, foreseeable and imminent risk to themselves or others." (National Association of Social Workers Code of Ethics 1.02 Self-determination)

The issue of informed consent is at the heart of the ethical principle of respect for the patient's autonomy. As the preface of Guideline C6 of the SMC's Ethical Code and Ethical Guidelines 2016 ("2016 ECEG") explains:

"An important part of patient autonomy involves ensuring that patients give their valid consent (if they are able) to any treatment or procedure prior to their undergoing such treatment or procedure. This involves the patients making voluntary decisions on their medical care after having known and understood the benefits and risks involved."

To address the case of Doctor L and the failure to obtain informed consent for a Percutaneous Coronary Intervention (PCI), we apply the **ETHIC Model** developed by **Elaine Congress**. This framework bridges the gap between clinical professional obligations and the fundamental right to patient autonomy.

E – Evaluate Values

- **Personal/Professional:** The doctor likely valued clinical efficiency or "beneficence" (acting in the patient's best interest to fix a cardiac issue). However, the SMC Ethical Code and Ethical Guidelines (ECEG) prioritizes **autonomy**—the patient's right to decide based on risk-benefit awareness.

- **Societal:** Society expects medical professionals to be experts but also demands transparency and the right to bodily integrity.

T – Think Relevant Laws and Case Decisions

- **The MRA & ECEG:** Under Section 53(1)(d) of the Medical Registration Act (MRA), professional misconduct includes breaches of the SMC 2016 ECEG.
- **Legal Precedent:** The High Court's ruling emphasizes that **record-keeping** (Guideline B3) is not just administrative; it is a legal safeguard for informed consent. The lack of a contemporaneous record in Doctor L's case made the "implied consent" defense legally untenable.

H – Hypothesize Possible Consequences

- **Action A (Proceed without explicit consent):** Results in professional disciplinary action, loss of patient trust, and legal liability (as seen with Doctor L).
- **Action B (Prioritize Autonomy/Documentation):** May delay the procedure slightly but ensures the patient is an active participant, protects the doctor from the Disciplinary Tribunal, and fulfills the "Dignity and Worth" principle.

I – Identify Who Benefits and Who is Harmed

- **Beneficiaries:** The patient benefits from **self-determination**, avoiding procedures they might have refused if risks were known. The profession benefits from maintained standards.
- **Harm:** The patient is harmed by a loss of agency (even if the medical outcome is "good"). The doctor is harmed by a conviction for **professional misconduct**. The most vulnerable—those who may not understand medical jargon—are most at risk when informed consent is bypassed.

C – Consult with Supervisor

- In a medical or social work setting, consultation should focus on the **Standard of Care**. A supervisor would insist on the use of a written consent form as "essential best practice" to meet the requirements of the 2016 ECEG Guideline C6.
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Summary Recommendation: To balance autonomy with professional obligation, the doctor must view **informed consent as a process**, not just a signature. Documentation must be "clear, legible, accurate, and contemporaneous" to satisfy the High Court's standards.

Under the **SMC 2016 Ethical Code and Ethical Guidelines (ECEG)**, valid consent is not a mere formality but a comprehensive process of ensuring patient self-determination.

1. Specific Criteria for "Valid Consent" (Guideline C6)

For consent to be considered valid under the 2016 ECEG, it must meet these core requirements:

- **Voluntariness:** The decision must be made freely, without coercion by the doctor, family, or insurers.
- **Capacity:** The patient must be able to understand, retain, and weigh the information to make a choice.
- **Adequate Disclosure:** Doctors must disclose **material risks** (those a reasonable patient would want to know) and risks specific to the patient's unique circumstances.
- **Understanding:** The doctor must ensure the patient actually understands the information, using interpreters if there are language barriers.
- **Documentation:** For complex or invasive procedures (like a PCI), consent must be **adequately documented**.

2. Where Doctor L's Process Failed

Based on the tribunal findings, Doctor L failed primarily in the **disclosure** and **documentation** phases:

- **Failure of Disclosure:** Doctor L failed to explain the specific risks and complications associated with the PCI (Percutaneous Coronary Intervention) before the procedure.
- **Lack of Documentation:** His medical records were found to be unsatisfactory, lacking the "clear, legible, accurate and contemporaneous" detail required by Guideline B3 of the 2016 ECEG.
- **Presumption of Consent:** He essentially bypassed the voluntary decision-making process, leading to a conviction for professional misconduct under the Medical Registration Act.

3. Possible Remedial Actions

To address these failings and prevent recurrence, the following actions are typically recommended:

- **Standardised Consent Forms:** Implement specific written consent forms for invasive procedures that list common material risks and alternatives.
- **Documentation Training:** The High Court indicated that proper record-keeping is now a high-priority expectation; doctors should undergo training on maintaining **contemporaneous notes**.
- **Communication Skills Upgrading:** Engaging in Continuing Medical Education (CME) focused on the "Montgomery" standard of care (patient-centric disclosure) to better identify what a "reasonable patient" would find material.
- **Systemic Review:** For clinics/hospitals, establishing a "time-out" or secondary check to verify that a signed, informed consent form is present *before* invasive procedures begin.

A written consent form would, therefore, be an essential best practice as part of documenting informed consent. Guideline C6 (2) of the 2016 ECEG requires that a physician must take valid and adequately documented consent from patients for tests, treatments or

procedures that are considered complex, invasive or have significant potential for adverse effects.

Record Keeping

The High Court found that Doctor L's record-keeping was unsatisfactory.

In light of the requirement in the 2016 ECEG that doctors "***must maintain clear, legible, accurate and contemporaneous medical records of sufficient detail***" (see guideline B3(1), 2016 ECEG),

The High Court gave indication of its expectation that the SMC would, in future cases, consider preferring charges for failure to keep proper records in similar cases.

In the case of **Doctor L**, the High Court and the Singapore Medical Council (SMC) drew a sharp line between clinical negligence (a civil wrong) and professional misconduct (a disciplinary breach).

1. Negligence: The Civil Standard

In medical law, negligence is typically assessed via the Modified Montgomery Test.

- **Focus:** It asks whether the doctor's failure caused **actual harm or loss** to the patient.
- **Remedy:** The goal is **compensation** (damages) for the victim.
- **Threshold:** A doctor might be negligent but not necessarily "guilty" of a moral or professional failing if the error was a simple, one-off lapse in judgment.

2. Professional Misconduct: The Disciplinary Standard

Under Section 53(1)(d) of the Medical Registration Act (MRA), the High Court clarified that professional misconduct is about the **standing of the profession**.

- **The "Lowman" Test:** Misconduct is defined as conduct that would be reasonably regarded as "disgraceful or dishonourable" by professional brethren of good repute and competency.
- **Beyond Harm:** Unlike negligence, a doctor can be convicted of misconduct even if **no physical harm** occurred. In Doctor L's case, the "harm" was the infringement of the patient's **autonomy**.

- **Intent and Gravity:** Misconduct involves a "serious negligence" or a persistent disregard for the 2016 ECEG. Doctor L's failure to document and obtain consent was viewed as a breach of a **fundamental ethical duty**, elevating it from a simple error to misconduct.

Key Distinctions in Doctor L's Case

Feature	Negligence	Professional Misconduct
Primary Goal	Compensate the patient for injury.	Uphold professional standards and public trust.
Crucial Element	Causation of harm/damage.	Breach of the SMC Ethical Code.
Documentation	Poor records might be "evidence" of a mistake.	Poor records (Guideline B3) are an independent basis for a charge.
Result	Monetary payout.	Censure, fine, suspension, or removal from the register.

The High Court's Warning: The Court signaled that failing to keep **contemporaneous records** is no longer just a "minor administrative slip"—it is a professional failure that the SMC should charge as misconduct to protect the integrity of the healthcare system.

Reference

[Ethical Code and Ethical Guidelines and Handbook on Medical Ethics | Singapore Medical Council](#)

Distributive Justice

In the context of the American Counseling Association (ACA), distributive justice is a core moral principle used to guide the fair allocation of resources and services. It ensures that ethical decision-making extends beyond individual client care to broader societal fairness.

Distributive Justice in the ACA Context

Distributive justice focuses on the "fair, equitable, and appropriate distribution" of benefits and burdens within a community.

- **Equality vs. Equity:** It mandates treating equals equally while also recognizing that "unequals" may require different treatment based on specific needs to achieve true fairness.
- **Resource Allocation:** In counseling, this often involves decisions about who receives treatment, how long they receive it, and how scarce resources (like sliding-scale slots or community grants) are distributed.
- **Social Advocacy:** The [ACA Code of Ethics](#) encourages counselors to advocate for systemic changes that promote justice and remove barriers to mental health services for marginalized populations.

The ACA Ethical Decision-Making Model

When a counselor faces a dilemma where justice or resource distribution is at stake, the ACA suggests a systematic process—often based on the [Forester-Miller and Davis model](#)—to reach a resolution:

1. **Identify the Problem:** Clearly define the issue and determine if it is legal, ethical, or professional in nature.
2. **Apply the ACA Code of Ethics:** Review the code to see if specific standards (e.g., Section A.11.b. on termination or referral) apply.

3. **Determine Nature and Dimensions:** Consider the six moral principles: **autonomy, nonmaleficence, beneficence, justice, fidelity, and veracity.**
4. **Generate Potential Actions:** Brainstorm multiple solutions without initial judgment.
5. **Consider Consequences:** Evaluate the potential impact of each action on all parties involved.
6. **Evaluate the Selected Action:** Apply tests of justice (would you treat others the same?), publicity (would you want this in the news?), and universality (would you recommend this to a peer?).
7. **Implement the Action:** Execute the plan and reflect on the outcome to improve future practice.

In this case study, we apply the [Forester-Miller and Davis \(1996\) model](#)—the framework recommended by the American Counseling Association (ACA)—to a dilemma involving **distributive justice**.

Case Details: The Scarcity Dilemma

- **The Setting:** A community mental health agency with a six-month waiting list.
- **The Conflict:** The agency has one open slot for intensive, weekly counseling. Two potential clients are referred simultaneously:
 - **Client A:** A high-functioning professional seeking help for moderate "burnout" who can pay the full fee.
 - **Client B:** An unemployed single parent experiencing severe depression and housing instability who requires a sliding-scale (low-cost) slot.
- **The Ethical Issue: Distributive justice**—how to fairly allocate a scarce resource (the counselor's time and the agency's limited low-cost capacity).

Potential Courses of Action (COA) and Consequences

Course of Action

Potential Consequences

COA 1: First-Come, First-Served (Admit whoever called first)	Pros: Simple, avoids bias. Cons: Fails to account for acuity of need or systemic barriers (injustice).
COA 2: Maximize Revenue (Admit Client A)	Pros: Supports agency financial health. Cons: Perpetuates inequity; Client B may spiral into crisis without intervention.
COA 3: Prioritize High Acuity (Admit Client B)	Pros: Aligns with social justice; prevents severe harm (Beneficence/Nonmaleficence). Cons: Agency may face "burnout" of low-cost slots if revenue isn't balanced.

Evaluation of the Selected Course of Action

The counselor chooses **COA 3 (Prioritize High Acuity)** based on the ACA's definition of justice: **"treating equals equally and unequals unequally but in proportion to their relevant differences"**.

- **Test of Justice:** Would the counselor treat others the same? Yes, in cases where the severity of the mental health crisis outweighs the ability to pay, the clinical need is the "relevant difference".
- **Test of Publicity:** Would the counselor be comfortable with this decision being public? Yes; prioritizing a client in crisis over a client with higher financial means but lower acuity aligns with the [ACA's commitment to social advocacy](#).
- **Test of Universality:** Would this be recommended to other counselors? Yes; it upholds the principle of **beneficence** (doing good) by addressing the most urgent risk first.

Selected Action: Enroll Client B. To mitigate harm to Client A, the counselor provides a **referral** to a private practitioner who has immediate availability for full-fee clients, ensuring Client A still receives care.

Incident Reports and Reporting Procedures

A client was hit by the carton boxes of diapers along the hallway in this nursing home. You are assigned to investigate the incident.

Devise Incident reporting and writing requirements: Filling out an Incident Report

Include the following information:

- The exact time and date
- The names of persons involved and any witnesses
- Factual information about what happened
- Other relevant facts, including your actions such as notifying the Manager/Director) and any corrective actions taken
- Follow your organization's policy regarding the policies and procedures for how the incident report is to be reported and followed up; this will determine whether the matter needs further investigation.
- After completing the incident report, you have to sign and date it.

How to write Incident Report

- **Write objectively.** Describe exactly what you saw. If you did not see the client fall, document that you found the client lying on the floor. Then document your actions, such as assessment of the client for injury, assisting the client back to bed and calling the doctor.
- **Incorporate client and witness accounts of the event into the report.** State their comments as direct quotes. Have the witnesses assist you in preparing the report and co-sign the final report.
- **Do not assign blame.** Refrain from pointing your finger at a co-worker or your organization's administration. An incident report is not an opportunity for you to point out inferior equipment or poor staffing.
- **Avoid hearsay and assumptions.** If your client is injured in another department, it is up to the person who witnessed the incident in that department to write the incident report.

Forward the report to the person designated by your organization's policy.

To investigate the incident where a client was struck by falling diaper cartons, we must align with the **Agency for Integrated Care (AIC)** guidelines and the **SMC/MSF** standards for safety and documentation.

1. Investigation & Standard Operating Procedures (SOP)

The investigation follows a structured **Incident Reporting Process**:

- **Immediate Response:** Ensure the client is medically assessed. Check for injuries and call the attending doctor or nurse if needed.
- **Fact-Finding:**
 - **Direct Observation:** Document the exact location and state of the hallway (e.g., "Three cartons of diapers found tilted in the Level 2 hallway").
 - **Interviews:** Collect direct quotes from the client and witnesses. *Example: Client stated, "I was walking past when the stack just tipped over."*
 - **Review Documentation:** Check if the boxes were placed there by a vendor or staff and if it violated fire safety or "clear hallway" protocols.
- **Reporting Requirements:** Complete the AIC Incident Reporting Form within **two working days**.
 - **Objective Writing:** Avoid hearsay. Do not write "the staff was lazy"; write "the cartons were stacked four-high without tethering."
 - **Co-signatures:** Ensure all witnesses review and co-sign the report to verify accuracy.
- **Notification:** Escalate to the Manager/Director immediately as per the organization's **Quality & Safety Standards**.

2. Analysis of Client's Rights Breach

In this case, the client's rights have been **breached** regarding **Beneficence** and the **Right to a Safe Environment**.

- **Why?** Under the NCSS Service Standards, agencies are mandated to provide a safe environment. Failing to secure heavy items in a common hallway is a breach of the duty of care.
- **Autonomy vs. Safety:** While the client has the right to move freely (autonomy), the organization failed in its **professional obligation** to prevent foreseeable harm (non-maleficence).

3. Ensuring Rights and Good Governance

To uphold client rights and ensure good governance to minimize recurrence, the following steps are required:

A. Upholding Client Rights:

- **Transparency:** Inform the client/family of the investigation findings and the corrective actions taken.
- **Advocacy:** Ensure the client has access to a feedback mechanism or an ombudsman if they feel the incident was not handled fairly.

B. Good Governance & Prevention:

- **Hazard Identification:** Conduct a weekly **Environment of Care** walk-through to identify storage hazards (e.g., "no-storage" yellow lines in hallways).
- **Training:** Conduct mandatory refreshers on **Workplace Safety and Health (WSH)** for all staff and vendors regarding the proper stacking and storage of supplies.
- **Policy Review:** Update the **Standard Operating Procedures (SOPs)** for logistics to ensure that high-volume items (like diapers) are moved immediately to secured storage rather than being left in transit areas.
- **Audit & Compliance:** Maintain a digital **Incident Log** to track trends. If "hallway obstructions" reappear, trigger an internal audit to comply with MOH Serious Reportable Event standards.

Here is a sample **Objective Narrative** drafted according to the SMC Ethical Guidelines and the AIC reporting standards.

Incident Report Narrative

Date of Incident: 23 October 2025

Time of Incident: 10:15 AM

Location: Level 2 North Wing Hallway, adjacent to Room 204

Persons Involved: Mdm. Tan (Client), Sarah Lim (Nursing Aide/Witness)

Factual Description of Event:

At 10:15 AM, Mdm. Tan was ambulating with her walker along the Level 2 North Wing hallway. A stack of four diaper carton boxes, positioned against the left wall, tilted and made contact with Mdm. Tan's right shoulder and arm. Mdm. Tan remained upright but ceased walking.

Staff Nurse (SN) Arul arrived at the scene at 10:17 AM. Upon arrival, SN Arul observed three carton boxes lying on the floor and Mdm. Tan leaning against her walker.

Client and Witness Statements:

- **Mdm. Tan stated:** "I was walking to the dining hall when the boxes just slid and hit my side. It gave me a fright."
- **Sarah Lim (Witness) stated:** "I was coming out of Room 205 when I saw the top two boxes slide off the stack. They struck Mdm. Tan on her right side."

Actions Taken:

1. **Immediate Assessment:** SN Arul performed a physical assessment. No bruising, skin tears, or swelling were noted on the right shoulder or arm at 10:20 AM. Mdm. Tan reported a pain scale of 2/10.

2. **Safety Measures:** Mdm. Tan was assisted to her room for a period of rest and further monitoring.
3. **Medical Review:** Dr. Wong was notified at 10:45 AM and scheduled a review for 2:00 PM to rule out delayed musculoskeletal injury.
4. **Environmental Correction:** The boxes were immediately moved to the secured Level 2 supply closet. The hallway was cleared of all remaining obstructions.
5. **Notification:** The Director of Nursing (DON) was briefed at 11:00 AM.

Corrective Actions & Follow-up:

The floor supervisor has initiated a review of the "Supply Delivery SOP" to ensure all vendors and staff transport items directly to storage zones. A hazard report was submitted to the Facilities Management team to mark "Keep Clear" zones in high-traffic hallways.

Evaluation of this Narrative (ETHIC Model Apply)

- **Objectivity:** The report avoids words like "careless" or "negligent." It describes the **physical movement** of the boxes and the **specific actions** of the staff.
- **Documentation Standards:** It fulfills the 2016 ECEG B3 Requirement by being contemporaneous and detailed.
- **Rights-Based:** By including Mdm. Tan's direct quotes, the report respects her **autonomy and voice** in the investigation process.

Final Signature and Verification

To finalize the report in compliance with SMC documentation standards and AIC quality protocols, the document must be formally verified.

Reporting Officer:

Signature: _____

Name/Designation: [Your Name/Investigator]

Date/Time: 23 Oct 2025, 4:00 PM

Witness Verification:

Signature: _____

Name/Designation: Sarah Lim (Nursing Aide)

Date/Time: 23 Oct 2025, 4:10 PM

Handling Witness Refusal to Co-Sign

If a staff member or witness refuses to sign the report—often due to fear of reprisal or misunderstanding the report's purpose—you must handle the situation with **professionalism and ethical clarity**.

1. Diffusing the Situation (De-escalation)

- **Clarify Purpose:** Explain that the report is a **legal and ethical requirement** for client safety, not a disciplinary tool. State: *"The goal is to prevent Mdm. Tan or others from being hurt again, not to assign blame to individuals."*
- **Review the Content:** Invite the witness to read the narrative again. Ask: *"Is there any specific sentence you feel is inaccurate?"* Offer to amend the narrative if their concern is about a factual error.
- **Address Fears:** Remind them of the organization's **Whistleblowing or Non-Retaliation Policy**, which protects staff who report incidents in good faith.

2. Ethical and Administrative Procedure for Refusal

If the witness still refuses to sign despite de-escalation:

- **Do Not Coerce:** Forcing a signature violates the principle of **veracity** and professional ethics.
- **Document the Refusal:** In the witness signature block, write: **"Witness [Name] declined to sign."**
- **File a Supplementary Note:** Prepare a brief, objective memo stating: *"On [Date], the draft report was shared with the witness. The witness confirmed the verbal account was accurate but declined to provide a formal signature."*
- **Third-Party Verification:** If another staff member was present during the initial verbal statement, they may sign as a witness to the **statement being given**, rather than a witness to the accident itself.

3. Governance Follow-Up

Report the refusal to the **Manager/Director**. This may indicate a "culture of fear" within the nursing home that requires **Good Governance** intervention, such as safe-reporting workshops or a review of the Personal Data Protection Act (PDPA) protocols to ensure staff feel their data is handled correctly.

To satisfy AIC Community Care quality standards and MOH Healthcare Performance requirements, a **Corrective Action Plan (CAP)** must address the root cause and implement systemic safeguards.

Corrective Action Plan (CAP): Incident #2023-10-25

Focus Area	Action Item	Responsibility	Timeline
Immediate Environment	"Clear Hallway" Mapping: Paint/Tape "Yellow Zones" in all transit corridors. No items may be placed in these zones for any duration.	Facilities Manager	Within 7 days

Operational SOP	Supply Chain Transition: Amend the Standard Operating Procedure (SOP) to require a "Two-Person Handover" for deliveries. Items must go from the truck directly to the storeroom.	Logistics Officer	Immediate
Staff Training	WSH Refresher: Conduct a mandatory "Workplace Safety & Health" briefing on the hazards of vertical stacking and corridor obstructions.	Nursing Lead	Within 14 days
Monitoring	Safety Rounds: Implement a "Daily Environmental Walkabout" checklist to be signed off by the Ward Manager at the start of every shift.	Ward Manager	Daily (Ongoing)

Preventing Potential Injuries (Root Cause Analysis)

To ensure **Good Governance**, we analyze the failure using the **ETHIC Model** logic:

1. **Engineering Controls:** Install wall-mounted shelving or tethers in storage areas to prevent "tilting" of light but bulky items like diaper cartons.
2. **Administrative Controls:** Update the **Risk Register** to include "corridor obstructions" as a high-frequency risk. This ensures the board of directors reviews safety compliance during quarterly meetings.
3. **Client Protection:** For clients with mobility aids like Mdm. Tan, staff must ensure pathways are clear *before* assisting them with ambulation, upholding the Dignity and Worth of the Person.

Verification of Effectiveness

The Agency for Integrated Care (AIC) requires evidence that the CAP worked.

- **Audit:** A surprise audit will be conducted in 30 days to ensure zero items are found in "Yellow Zones."
 - **Feedback:** Interview Mdm. Tan and other residents to confirm they feel the environment is safer and more accessible.
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To satisfy the **Good Governance** and **Quality & Safety Standards** required by AIC and the Ministry of Health (MOH), this checklist is designed to be a "living document" for daily accountability.

Daily Environmental Safety & Hazard Checklist

Ward/Area: _____ **Date:** _____ **Time:** _____

Checklist Item	Status (Pass/Fail)	Corrective Action Taken	Initials
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1. Hallways & Corridors: Are all "Yellow Zones" (Keep ☐ Clear zones) completely free of carton boxes, wheelchairs, or trolleys?

2. Storage Stability: Are supplies in storage areas ☐ stacked below shoulder height or properly secured to prevent tilting/falling?

3. Flooring & Surfaces: Are hallways free of spills, loose mats, or trailing electrical cords that could cause trips? ☐

4. Lighting: Are all transit areas and exit signs well-lit to ensure visibility for clients with visual impairments? ☐

5. Mobility Clearance: Is there a minimum of 1.2 meters of clear width in hallways for two walkers/wheelchairs to pass safely? ☐

6. Signage: Are "Wet Floor" signs available and used immediately upon any spill or cleaning activity? ☐

7. Equipment Parking: Are unused hoists and medication trolleys parked in designated bays, not in the hallway? ☐

Governance & Implementation Instructions

1. **Contemporaneous Recording:** In line with the SMC 2016 ECEG Guideline B3, this checklist must be completed at the start of every shift.
2. **Immediate Remediation:** If a "Fail" is noted (e.g., diaper boxes in the hallway), the staff member must move the items **immediately** and document it in the "Corrective Action" column.

3. **PDPA Compliance:** Ensure that if any client-specific hazards are noted, no sensitive personal data is visible on the publicly accessible checklist, adhering to the Personal Data Protection Act (PDPA).
4. **Escalation:** Any recurring hazards (e.g., boxes left by the same vendor daily) must be escalated to the **Manager/Director** for a formal review of vendor contracts.

Staff Name: _____ **Signature:** _____

Supervisor Verification: _____ **Date:** _____

To align with the **AIC (Agency for Integrated Care)** digital reporting standards, the manual hazard checklist should be transitioned into a **Digital Incident Management System (IMS)** using platforms like **FormSG**. This ensures data security, PDPA compliance, and the ability to track safety trends over time.

Integration into a Digital System (FormSG Approach)

1. Digital Submission Workflow

- **Prompt Reporting:** Instead of paper logs, staff complete the "Daily Hazard Checklist" via a mobile-friendly FormSG link. This creates a **time-stamped, contemporaneous record** that satisfies SMC 2016 ECEG Guideline B3.
- **Security:** Any incident involving client data (like Mdm. Tan's injury) must be uploaded as an **encrypted/password-protected file** within the FormSG portal to maintain confidentiality.
- **Timeline Automation:** The system can be set to trigger alerts if a checklist is not submitted by 09:00 AM daily, ensuring continuous **governance**.

2. Tracking Safety Trends (Data Analytics)

By digitizing the checklist, the nursing home can move from *reactive* reporting to *proactive* risk management:

- **Trend Identification:** Data from FormSG can be exported to identify "Hot Zones." For example, if "Hallway Obstructions" are flagged 80% of the time on Tuesdays (delivery days), the **Corrective Action Plan (CAP)** can specifically target Tuesday logistics.
- **Performance Dashboards:** Create a monthly safety scorecard for the **Board of Directors**. This demonstrates **Good Governance** by showing a reduction in "near-miss" incidents over time.

3. Integration with Official AIC Reporting

- **Seamless Escalation:** If a "Fail" on the daily checklist results in an actual injury (like the diaper box incident), the digital data is already formatted for the AIC Level 3 (High) Incident Report.
- **2-Day Requirement:** The digital system ensures the report is filed within the mandatory **two working days** timeline by providing automated reminders to the investigating officer.

Reference

NCSS Service Standards Framework

[Service standards](#)